Major Incident Therapies Operational Policy

Version 1

January 2026

Authors:	Jenna Hills	Karen Scott	Mark Lilley	Lynsay Stewart
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1. Major Incident Background

Definition: Any occurrence that presents serious threat to the health of the community, disruption to the service or causes (or is likely to cause) such numbers or types of casualties as to require special arrangements to be implemented by one or more territorial and/or special health boards simultaneously or in support of each other. It requires considerable resources and strategic input as it potentially threatens the survival of an organisation.

Once a major incident is declared there is a comprehensive major incident plan which will be followed, including action cards for all staff, which detail roles and responsibilities during the incident. The existing Major Incident plan covers the acute phase of an incident of admitting patients to the hospital, reuniting them with their families, and emergency interventions such as scanning and theatre. Once patients have been admitted to a ward the major incident plans ceases to provide further information regarding their ongoing treatment and rehabilitation.

2. Document Scope

This plan will detail the service provision by therapy staff and the adjustments required in order to deliver the interventions. This document will be actioned from the point where a Major Incident is declared, and will continue until rehab caseload has returned to normal capacity/business as usual. This could be days, weeks or months following the official stand down from a major incident and will depend on the rehabilitation need and level of intervention required by patients admitted during a major incident.

3. Rehab personnel

Major Incident Clinical Leads

Major Trauma Coordinators (MTC)

Major Incident Rehab Therapy Group members			
Paediatrics		**Adults**	
Role	**Contact number**	**Role**	**Contact number**
Major Trauma Coordinators	84931	Consultant AHP	83905
Physiotherapy Lead	84650	Senior Representative from Physiotherapy MT Team	82147
Occupational Therapy Lead	85678	Senior Representative from OT MT team	83907
Speech and Language Therapist	85785	Senior Representative from SLT MT team	86368

Dietetics Acting Lead	07789920948	Senior Representative from Dietetics MT team	86243
Child and Adolescent Consultant Psychiatrist, Paediatric Liaison Psychiatry	86529	Representative from Psychology MT Team	83908
Orthotics Lead	07720077015	Associate Chief/ Chief AHP/ MT Team Lead	83710/ 83712/82945
Physiotherapist	84804/84730/84800	Rehab Coordinators	TBC
Major Trauma Occupational Therapist	85737	Representative from Specialist Rehabilitation	
Clinical Service Manager	07779435530	Adult Mental Health Liaison Service, QEUH	0141 201 2422
Major Trauma Coordinators from each Health Board area as required		Major Trauma Coordinators from each Health Board area as required	
Neurorehabilitation Clinical Nurse Specialist	84885		

4. Initial Action following declaration of a Major Incident

Upon declaration of a Major Incident the Major Trauma Coordinators's (MTC) would alert therapy teams and immediately schedule a Rehab Professionals Group Multidisciplinary team meeting (MDT) to take place as soon as possible within core working hours. At this point the number of casualties admitted would be known and teams could action this plan accordingly.

Daily Rehab MDT meetings would be established and will continue until the rehab teams had returned to business as usual. MDT's would be held at 10am daily, with the potential for a second MDT at 4pm if required. For adult patients MDT's would be held in QUEH's Major Trauma ward seminar room. For paediatric patients, the MDT would be held in RHC in the 3rd floor 3D gym. In a mixed incident MDT's would be staggered to allow for cross cover across sites.

Patients would be discussed on a daily basis in terms of ongoing rehab requirement, resources available and optimal environment for the patient. As per normal practice an approach to early mobilisation would be utilised both for individual patient benefit and for critical care service resilience and recovery.

5. Rehab Teams Response

Major Trauma Coordinators (MTC)/Rehab Coordinators: Action Card

The role of the MTC's/Rehab coordinators is to coordinate and liaise with all teams involved in the care and treatment of patients admitted to hospital due to a major trauma. They liaise closely with patients and families from admission until discharge and act as a single point of contact throughout their admission. They help to establish the patients rehabilitation needs and then coordinate these needs with all the necessary and relevant teams.

Following a major incident the MTC's/ Rehab Coordinators will organise and chair daily rehab MDT's to ensure all rehab, medical and nursing needs are updated accordingly. These MDT meetings will support the identification of ongoing rehabilitation needs, facilitate discharge planning, and coordinate patient care. Additional medical and nursing specialties may be involved as required, depending on the patient's age, rehabilitation needs and ongoing interventions. The MTCs/Rehabilitation Coordinators will also attend daily ward rounds and act as a single point of contact for patients and their families throughout their admission.

Rehab plans would be started by the MTC's/ Rehab coordinators and would help determine suitability for repatriation and ongoing rehabilitation needs.

Major Trauma coordinators from the Trauma Units would be invited to attend daily MDT via MS Teams, to ensure good communication and facilitate seamless repatriation and discharge planning.

MTC	Adults	Paediatrics
WTE	5 x WTE Major Trauma Coordinators 2 x WTE Rehab Co-ordinators	3.0
Hours of work	MTC: 7.30am-7.30pm 7 days RC: 8.30am-4.30pm Mon-Fri	07:30 to 17:30 Monday to Friday Sunday 08:00 to 12 noon.
Contact	82149/82150	84930/84931/84845

Major Trauma MDT Coordinator

The MDT coordinator would assist with the organisation of all rehab MDT meetings, keeping records and contacting staff and families where required. They would ensure there was a written record of discussion points and goals and assist with booking of rooms, inviting members to attend and liaising with relevant teams and families.

Rehab Teams Response

Please note staff WTE ratios for adults are trauma specific allocated professionals. The Paediatric WTE ratios are for RHC rehab professionals hospital wide cover, as there is no dedicated Major Trauma ward in RHC unlike QEUH.

Speech and Language (SLT)

The Speech and Language Therapy (SLT) team provide assessment, advice, intervention and support regarding communication and eating and drinking and swallowing for patients involved with the Major Trauma service. The SLT team will be involved in making clinical decisions regarding recommendations for oral intake, modification of diet and fluids and eating/drinking/swallow rehabilitation. SLT are also involved in assessing communication and facilitating strategies and rehabilitation for patients who have a communication impairment following major incident. This may incorporate augmentative or alternative communication along with traditional rehabilitation approaches.

The SLT team's function following a major incident will include:

(1) Attendance at daily major incident rehab MDT meetings to support coordination of rehabilitation, identification of ongoing rehabilitation needs and discharge planning for this patient group

(2) Assessing and identifying communication and/or swallowing impairments as a result of major incident/trauma and providing support to guide oral intake, facilitate communication, direct rehabilitation and education to the patient and their family.

(3) SLT will also provide support for critical care patients including prolonged intubation tracheostomy with regard to weaning, swallow and communication as appropriate.

(4) The SLT will link in with local colleagues to ensure smooth transition of patients on repatriation to local Trauma Unit or discharge to community SLT services where clinically appropriate.

SLT	**Adults**	**Paeds**
WTE	3.6	4.9
Core Working Hours	Mon - Fri 8.30am-4.30pm	Mon-Fri 8.30am-4.30pm. No weekend cover.
Adjustment:	Reassign staff, cancel clinics	Reassign staff, cancel clinic, additional staff, extended hours
Maximum patient threshold:	24	10
Community links:	Yes, throughout West of Scotland Trauma Network	Yes throughout GGC.
Discharge issues:	Availability of suitable discharge location	Community f/u
Resolutions:	Link in with wider acute SLT service for support	
Previous strategies:		Cancelling of MDT clinic not SLT led.

Occupational Therapy (OT)

The Occupational Therapy team provide assessment, treatment and equipment provision to patients and their families involved in the major incident, as/when required, to facilitate rehabilitation and ensure safe discharge home from hospital. Interventions and assessments include, but are not limited to, assessment and provision of specialist seating and equipment, cognitive and post-traumatic amnesia assessment and support, and interventions to enable resumption of daily routines and activities of daily living (ADL).

The Occupational Therapy team's function following a major incident will include:

(1) Attendance at daily major incident rehab MDT meetings to support coordination of rehabilitation, identification of ongoing rehabilitation needs and discharge planning for this patient group

(2) Support campus wide response to facilitate discharge home of current inpatients to free up bed capacity if required

(3) Assessing and identifying functional difficulties with Activities of Daily Living (ADLs) due to physical and/or cognitive deficits as a result of major incident/trauma. This includes personal care, transfers domestic tasks and play as appropriate. Also facilitating return to employment and/or education and community living skills.

(4) The OT will link in with local colleagues to ensure smooth transition of patients on repatriation to local Trauma Unit or discharge to community OT services (Specialist Children's Services) where clinically appropriate.

OT	**Adults**	**Paeds**
WTE	7.8	7.2
Core Working Hours	Mon-Fri 8.30-4pm Reduced service provided at weekend 1 x WTE OT provides cover 1 day either (Sat/Sun) contracted as 5/7 working.	Mon-Fri 8.30am- 4.30pm. No weekend cover.
Adjustment:	Reassign staff from different specialities, potential extension of hours, consider additional number of staff working weekends	Extended hours and 7/7 cover if additional funding allocated. Cancelling clinics and reassign staff.
Maximum patient threshold:	24	5-10

Community links:	Could be explored to facilitate earlier discharge	Specialist children's services community staff, already one integrated service
Discharge issues:	Delays to Packages of care (PoC) starting, delays to equipment provision including wheelchair provision, delays to Community Rehabilitation Team input commencing	Equipment, time for house adaptations
Resolutions:	Equipment store onsite. Liaise with other sites to obtain equipment if required. Early referral for PoC and escalation to Senior Management team if delays.	Core stock of equipment, better links with joint equipment services out of GGC. Preparation of rolling stock for basic equipment provision.
Previous strategies:	Would pull staff from other acute areas, with appropriate support from MT team to allow community staff to be able to support patient discharges	Process packs to upskill community staff to assist acute staff, this can be relaunched after review and refresher training.

Physiotherapy (PT)

The role of the Physiotherapy service is to provide early, coordinated intervention to improve the respiratory status and reduce the sequelae of immobility, in addition to providing rehabilitation as part of the wider MDT to promote functional ability and facilitate safe discharge home.

Following a major incident the Physiotherapy Service will prioritise patients with respiratory requirements, provide appropriate physiotherapy rehabilitation, establish future physical rehabilitation needs, liaise with community physiotherapy services and ensure patients are physically safe for discharge with appropriate walking aids. The physiotherapy team will work closely with the MTCs and MDT and will attend any meetings as required.

The physiotherapy team's function following a major incident will include:

(1) Attendance at daily major incident rehab MDT meetings to support coordination of rehabilitation, identification of ongoing rehabilitation needs and discharge planning for this patient group

(2) Support campus wide response to facilitate discharge home of current inpatients to free up bed capacity if required

(3) Assessing, identifying and treating complex physical rehabilitation needs to promote functional independence which are as a result of major incident/trauma.

(4) The PT will link in with local colleagues to ensure smooth transition of patients on repatriation to local Trauma Unit or discharge to community PT services where clinically appropriate.

PT	**Adults**	**Paeds**
WTE	9.33	WTE 28/37
Core Working Hours	Mon – Fri 8.30am – 4pm Reduced service provided Sat and Sun 8.30am - 4.30pm by 2 x PT. Contracted as 5/7 working. Emergency Physiotherapy respiratory on call service available outwith normal working hours evenings and weekends	Mon-Fri 8.30am to 4.30pm, there is an on call service in place for evenings and weekends.

Adjustment:	Reassign staff from different specialities, potential extension of hours, consider additional number of staff working weekends	Cancelling clinic, 7/7, extended hours, reassign staff, additional staff
Maximum patient threshold:	24	5
Community links:	Could be explored to facilitate earlier discharge	Could be explored.
Discharge issues:	Delays to Packages of care (PoC) starting, delays to equipment provision, delays to Community Rehabilitation Team input commencing	Community f/u, equipment, housing issues
Resolutions:	Equipment store onsite. Liaise with other sites to obtain equipment if required. Early referral for PoC and escalation to Senior Management team if delays.	
Previous strategies:	Would pull staff from other acute areas, with appropriate support from MT team to allow community staff to be able to support patient discharges	Community physiotherapy services supporting into the acute. Possible cross cover from adult services if paediatric specific incident.

Dietetics (DIET)

Nutritional support is an integral component in the care of major trauma patients and needs to be continuously monitored in the sub-acute phase and the rehabilitation phase as nutritional deficits are associated with longer hospital stays and higher rates of post-operative complications and mortality.

All patients admitted to intensive care with major trauma will have a dietetic assessment and require nutritional support in the form of enteral nutrition and/or parenteral nutrition. Their nutritional needs will be monitored and reviewed daily, as required. On step down from ICU/PICU and transfer to a general ward the nutritional care plan will be reviewed by the ward Dietitian.

All other patients should have a nutritional screening within 24hours of admission/transfer from ICU/PICU using a validated nutritional screening tool; this will facilitate early intervention and identify if a patient is at risk of nutritional deterioration.

Any patients requiring ongoing support with their nutrition will be transferred to the community dietetic team for continued review.

RHC specific: Infants and children are more vulnerable to nutritional deterioration/deficits due to the high requirements for growth.

The dietetic team's function following a major incident will include:

(1) Attendance at daily major incident rehab MDT meetings to support coordination of rehabilitation, identification of ongoing rehabilitation needs and discharge planning for this patient group

(2) Support campus wide response to facilitate discharge home of current inpatients to free up bed capacity if required

(3) Assessing, identifying and treating nutritional needs of individuals as a result of major incident/trauma and providing education and support to maximise appropriate oral intake and /or artificial feeding to facilitate recovery and rehabilitation for the patient and their family.

(4) The dietitians will link in with local colleagues to ensure smooth transition of patients on repatriation to local Trauma Unit or discharge to community dietetic services where clinically appropriate.

DIETITIANS	**Adults**	**Paeds**
WTE	1.5	31 (includes community staff)
Core Working Hours	Mon-Fri 8.30am – 4.30pm	Mon-Fri 8.30am to 4.30pm. On Call Sat and Sun am for urgent inpatients. 7 day service for special feeds.

Adjustment:	Seek guidance and support from speciality specific dietetic teams	Cancelling clinics, reassign staff
Maximum patient threshold:	24	10 (however no current funding for Trauma)
Community links:	Onward referral to appropriate community dietetic teams/use of Community pharmacy nutrition support services as required	Dietetic service includes acute and community workload so that would be focus of reworking workload
Discharge issues:	Equipment and nutritional product supplies, community follow up	Equipment, community follow up, clinic appointments
Resolutions:	Seek guidance and support from speciality specific dietetic teams to support enhanced triaging and prioritisation	Additional staffing. Additional support for special feeds production Management of workflow may have to divert to registered dietitians to release support workers and supervisors. It may be possible to liaise with catering colleagues to support delivery of items to wards if required.
Previous strategies:	Use of first line advice and digital signposting	Divert and increase digital

Major Trauma Mental Health Services

The provision of psychological and psychiatric support following a Major Incident will involve co-ordinated involvement of several acute mental health services, with the nature of each service's involvement depending on the patient's injury profile (e.g. if brain injury present), mental status and clinical risk. Services involved will include:

Paediatric: Paediatric Liaison Psychiatry; Paediatric Neuropsychology (NP); Paediatric Clinical Psychology (CP).

Adult: Adult Mental Health Liaison Service; Major Trauma Clinical Neuropsychology

All patients involved in the Major Incident will have their psychological and psychiatric needs assessed, with advice and/or treatment of emotional, cognitive or behavioural difficulties resulting from their major traumatic injuries tailored accordingly. Psychological support and/or signposting will also be offered to patient's families. The Major Trauma Mental Health Services will also provide psychological first aid to patients and their families as/when required. This may be directly or indirectly via supporting the wider MDT deliver this support.

Psychological First Aid is described as "a humane, supportive and flexible response to a fellow human being who is suffering during or after a crisis". It is comprised of a number of components detailed in Appendix 8 of the 'Preparing for Emergencies Guidance' (Scottish Government, November 2023). If input beyond this provision is required the relevant Major Trauma Mental Health service will undertake more in-depth assessment and treatment of identified cognitive, emotional or behavioural difficulties to reduce psychological distress, promote wellbeing and support the patient to engage in rehabilitation to optimise recovery.

The major trauma mental health team's function following a major incident will include:

(1) Attendance at daily major incident rehab MDT meetings to support coordination of the psychological care and treatment for this patient group, including identifying any potential needs or vulnerabilities within patients and/or families that require psychological/psychiatric treatment following a major incident, either presently or in the future.

(2) Assessing, identifying and treating (as/when appropriate) the emotional and cognitive needs of individuals following a major incident/trauma and providing education, support and signposting to reduce psychological distress, promote wellbeing and facilitate recovery and engagement in rehabilitation for the patient and their family.

(3) Co-ordinate with staff in local hospitals on patient's repatriation or relevant community and third sector services where clinically appropriate to ensure ongoing provision of appropriate psychological and/or psychiatric support either acutely or in the future.

(4) Provide support to QEUH and RHC Major Trauma staff working with this patient group by offering consultation, de-briefs and also via liaising with the existing wider staff wellbeing services to provide appropriate signposting and ongoing support where required.

QEUH specific:

Ensure all patients have access to psycho-education resources for management of trauma. Provide specialist assessment where there is concern about significant psychiatric sequelae of trauma and advise on appropriate follow-up.

In the event of a Major Incident AMHLS will make urgent assessment of patients who may be suitable for discharge but may require community psychiatric follow-up or transfer to a psychiatric inpatient unit.

RHC specific: Ensure all young people and families have access to psycho-education resources on early management of trauma as part of psychological first aid. Appropriate psycho-education materials include: 'After the Event' by David Trickey; materials from the Anna Freud website and 'Supporting Children and Young People involved in a major trauma: Advice for parents, caregivers and teachers' (British Psychological Society)

Mental health	**Adults**		**Paeds**
	MT Neuropsych	**Psychiatry**	
WTE	2.5 WTE		
Core Working Hours	Mon-Fri 8.30am – 4.30pm	All services work Mon-Fri 9am – 5pm If OOH, MTU can access advice from 2nd on-call Psychiatry SpR for South Glasgow via Leverndale Hospital Switchboard.	All services work Mon-Fri 9am-5pm. **If OOH, please follow usual OOH procedures to access emergency psychiatric assessment and/or advice if required -** All initial calls should be directed to Mental Health Assessment Unit switchboard (0141 201 3136) for further allocation unless it is a paediatric inpatient referral (under 12 years of age) in which case RHC switchboard (0141 201 0000) should be contacted who will direct to the 2nd on call doctor.
Adjustment:	Re-configure allocated staff resource within MT from other aspects of service provision to prioritise clinical	Adult Mental Health Liaison Service Staff will be contactable 9am – 5pm Mon – Friday on 0141 201 2422.	Our services have identified a Core Group of clinicians who will be contactable via Paediatric Liaison Psychiatry contact number 0141 451 6529 (internal

	contact. Liaise with clinical leads within wider Regional Neuropsychology service and/or Acute Clinical Health psychology if required.	Non-urgent clinical activity will be postponed if required during a Major Incident	86529) within working hours (Monday – Friday, 9am – 5pm). Cancelling non urgent clinical activity if required however, this may not be required.
Maximum patient threshold:	24	Unknown, as would vary depending on other clinical activity and what requires from an acute response.	Unknown, as would vary depending on other clinical activity and what requires from an acute response.
Community links:	Could be explored if required	AMHLs will liaise with patient's CMHTs and crisis teams as appropriate	Potentially, however, they would likely not require to be utilised to support on site, it may be that the Tier 3 CAMHS Team or other mental health services are impacted in the months following an MI given the potential need to access services following discharge.
Discharge issues:	If high cognitive rehabilitation needs requiring specialist rehabilitation, likely to struggle with capacity resulting in prolonged wait for admission.	AMHLS will consider in each case whether crisis team intervention, CMHT follow-up, or transfer to psychiatry inpatient unit is indicated. AMHLS can also signpost to third sector sources of support.	Only if a high level of psychological need is present which requires prolonged mental health admission etc. Otherwise little challenges for discharge have been identified
Previous strategies:	Potential use of cross-site cover to support staffing within Neuropsychology services. Potential use of NHS GG&C psychological/ wellbeing support services	Consider support required for staff during a Major Incident	Staff wellbeing support and potentially providing additional support services for staff.

The RHC Acute Mental Health Services have also developed a comprehensive document on how to ensure a cohesive team approach to a major incident. The link to this document is in Appendix 2.

Orthotics (OR):

The Orthotic team provide immediate trauma care and ongoing medium and long term rehabilitation as required.

The initial trauma care can include assessment for bracing to facilitate fracture protection, immobilisation, maintenance of range of motion and facilitation of continued mobility. The Orthotic Rehabilitation Technicians also provide the Short Term Loam Wheelchair Service within RHC facilitating discharge by aiding early mobility.

The medium and long term follow up can include local and out of area patients and facilitation of transfer of care to their local area as required. It can also include ongoing care for specialist treatment e.g. Spinal bracing, RGO, complex orthotic prescriptions.
The goal is to maximise recovery from injury.

ORTHOTICS	**Adults**	**Paeds**
WTE	1.5 x WTE	15
Core Working Hours	Mon- Fri 8.30am to 4.30pm	Mon-Fri 7.30am to 5:00pm. No on call service.
Adjustment:	Cancelling clinics, extended hours, reassign staff	Cancelling clinics, extended hours, reassign staff
Maximum patient threshold:	24	5
Community links:	Acute team see GGC patients as outpatients as required. Good links with community services in other Health Boards	Acute team provides the community service.
Discharge issues:	Time to deliver treatment and time to await delivery of specialist items	Time to deliver treatment.
Resolutions:	Bulk stock of off the shelf products. Custom products have short delivery timeframe as agreed with supplier/manufacturer	Good planning and adequate resources
Previous strategies:		

Paediatric Neurorehabilitation Clinical Nurse Specialist (CNS):

The role of the paediatric neurorehabilitation clinical nurse specialist is to provide expert advice and support to patients involved within the paediatric major trauma service. The CNS will assist with the assessment, planning, implementation, evaluation, and review of patient care needs. The CNS will be responsible for the development, planning and co-ordination of the discharge plan and implementing the discharge timescales in consultation with the multidisciplinary team. The CNS will provide support, information and detailed advice on life-style changes to patients involved in the major trauma service and their families and information on long-term implications as a result of acquired brain injury or spinal cord injury.

6. Maximum Threshold

Teams have individually assessed what their threshold would be before they would require to make adjustments to normal service. The numbers are documented in section 5. The adult service would be able to manage up to 24 patients before the threshold would be breached, however it would be impacted by how many Major Trauma patients were already in-patients in the Major Trauma ward pre-incident.

Following initial admission and any immediate interventions for patients, the rehab MDT would assess if the threshold for rehab intervention has been breached. This would depend on several factors:

- Pre Major Incident numbers of current Major trauma in-patients
- Level/complexity of rehab required by existing and new patients from major incident
- Range of rehab services required
- Available resources

The relevant flowcharts should be utilised to determine if movement of patients is indicated. See Appendix 3 and 4 for flowchart demonstration. Specialist rehab staff would determine at MDT discussion a dynamic assessment of rehab needs.

7. Adjustment Options and Support Measures

Each team have considered what strategies they could employ to allow for an influx of patients. The options are as follows:

- Extended working hours or staggered shift patterns
- Temporary 7 day service, or enhancement of existing service
- Reassign/reallocation of staff
- Additional staff (utilising community links)
- Cancellation of clinics or outpatient appointments

Some of these options would require additional funding. Management have given theoretical permission for cross cover, overtime authorisation etc. Cancellation of any services would be temporary and adjusted according to caseload demands.

8. Mutual Aid

If the incident only involves paediatric patients or only involves adult patients there is the option to provide cross cover support. If adult rehab threshold is breached but patients require ongoing Major Trauma Centre level rehabilitation at QEUH then in addition to the above measures RHC staff would assist in delivering rehab sessions with guidance from adult major trauma therapy staff. This would be replicated if the patients were paediatric, QEUH major trauma therapy staff

would assist in delivering rehabilitation to paediatric patients with guidance from RHC therapy staff. (See Appendix 3 and 4)

9. Established links with community

The Major Trauma coordinators within QEUH and RHC only cover the Glasgow area, however there is a strong network of MTC's covering the whole of Scotland who could provide support where necessary. The MTC within each Trauma Unit will support with signposting and liaising within their Health Board area.

RHC links: Both SLT and Physiotherapy acknowledge their existing community links, which could be explored if required for additional workforce, or support as needed.

Within paediatrics, several services already act as both the acute and community service for their profession. Dietetics and orthotics staff already provide ongoing care requirements for patients within the community. Similarly Occupational Therapy is already an integrated service and have established links. The dietetics service stated that they already have a large community caseload and any redistribution of workforce would require both acute and community staff, however they have no dedicated funding for Trauma patients.

Paediatric Neuropsychology does not have a community counterpart, so there are no opportunities for support or onward referral at present. Liaison Psychiatry have potential links, but would unlikely utilise them acutely on site. It would be more useful to access them in the months following discharge from RHC.

10. Discharge challenges

The Major Trauma Coordinators would utilise support from the other MTC's throughout the Scottish Trauma Network with these established links with other health boards utilised where required following a major incident. These links would be particularly beneficial regarding repatriation back to residing health boards and the MTC's would facilitate a smooth transfer and oversee communication with all the various therapy, medical and nursing teams involved in order to ensure the patient's ongoing care needs are met.

Rehab Teams identified the following common themes as issues for discharge: equipment, community follow up, house adaptations, clinic appointments and other. It was felt equipment and community follow up were the most likely to present as challenges.

Within the other category; time to deliver treatment, school/workplace reintegration and repatriations were noted as potential areas for complications following a major incident.

These challenges would require support from management both financially and with provision of additional resources when required.

11. Actions to limit discharge complications

- Establish daily rehab MDT for all rehab teams, medical and nursing.

- Early contact with local rehab services and local Major Trauma coordinators. Identify ongoing rehab needs from daily rehab MDT and determine staff to contact teams at early stage of rehab.
- MTC to ensure good level of communication across both acute, local and community teams. Ensure hybrid approach to daily rehab MDT where local Major Trauma coordinators can attend via MS Teams.
- Provide notice where possible to patient transport of likely dates of discharge for patients and volume of patients.
- OT staff to establish core stock of equipment, explore potential for joint equipment services out of GGC. Preparation of rolling stock for basic equipment provisions.
- Dietetics may require additional support within special feeds production unit. Restructure of registered dietitians to release support workers and supervisors. Possibility of liaising with catering colleagues to support delivery of items to wards.
- Neuropsychology are likely to feel the impact of a major incident in the months following, when children return to normal activities and require neuropsychological assessment/intervention. Screening and triage would be required to mitigate this.

12. Non-MI patients

- Service would continue to non-MI patients in as normal a capacity as possible. Ongoing therapy input to intensive care and ward based patients would continue, particularly to facilitate safe and timely discharge.

Appendix 1a: RHC Action Card for MTC staff

RHC Major Incident Therapy Action Card

Action Card 1a	**Major Trauma Coordinator**
	Contact Number 84930/84931/84845

Role and Responsibilities

- To liaise with families and act as a single point of contact.
- To coordinate care from admission to discharge with medical, nursing and rehab teams.
- Communicate with external teams such as police, social work and education

Notified by	Report to
Senior Nurse on Duty (85770/Page 18502)	Medical and Nursing Teams for each patient.

		Completed
1.	Establish communication with all rehab teams	
2.	Identify named representative from each team who will attend daily MDT and advise time and location of daily rehab MDT	
3.	Chair initial rehab MDT to provide details to all teams on: number of patients, injuries, ward location and likely rehabilitation needs	
4.	Continue to hold daily MDT's. Twice daily if required.	
5.	Act as single point of contact for families throughout admission.	
6.	Assist with discharge planning and onward care	
7.	Coordinate any repatriation of patients to other acute sites	
8.	Discontinue MDT's with agreement from therapy group once caseload within normal capacity.	

Appendix 1b: QEUH Action Card for MTC staff

QEUH Major Incident Therapy Action Card

Action Card 1b	**Major Trauma Coordinator**
	Contact Number 82149/82150

Role and Responsibilities

- To liaise with families and act as a single point of contact.
- To coordinate care from admission to discharge with medical, nursing and rehab teams.
- Communicate with external teams such as police, social work and education

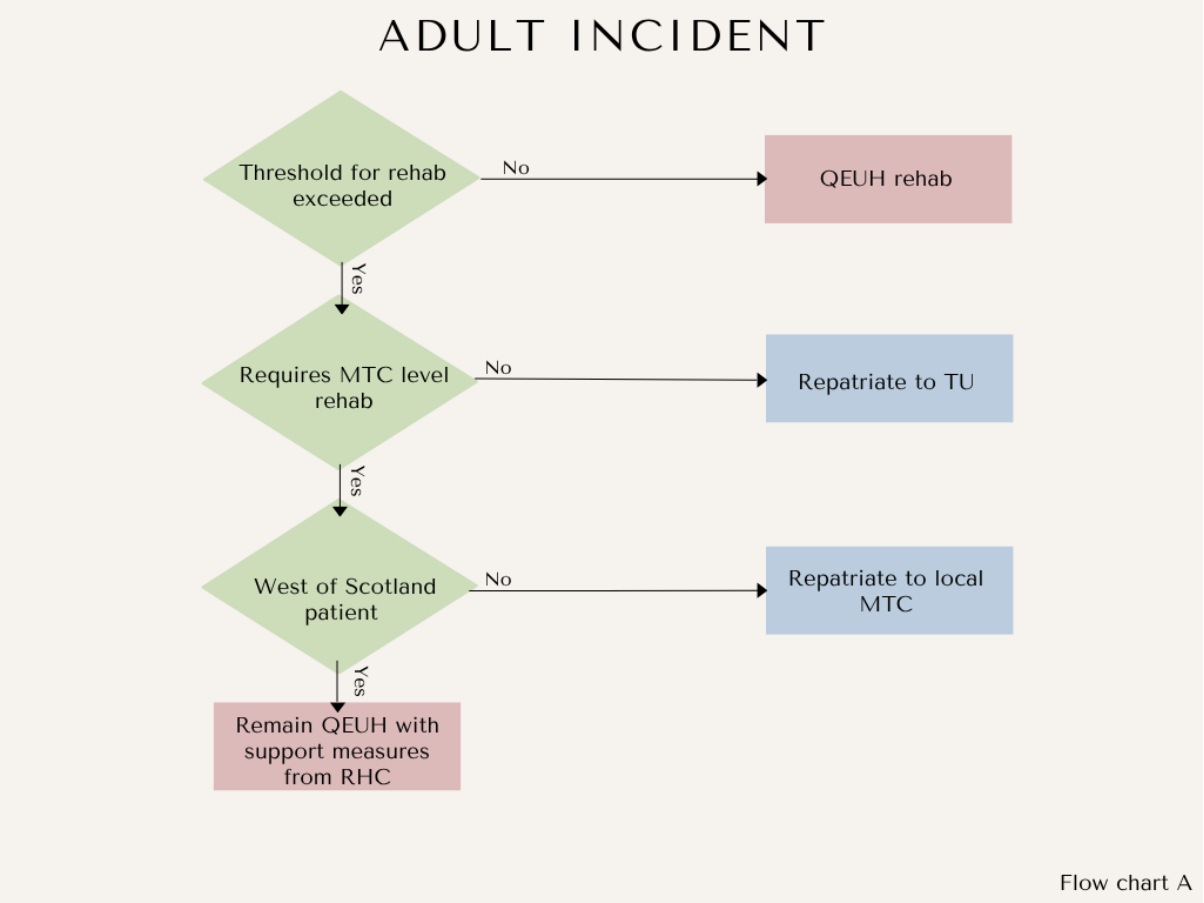
Notified by	Report to
Senior Nurse on Duty	Medical and Nursing Teams for each patient.

		Completed
1.	Establish communication with all rehab teams	
2.	Identify named representative from each team who will attend daily MDT and advise time and location of daily rehab MDT	
3.	Chair initial rehab MDT to provide details to all teams on: number of patients, injuries, ward location and likely rehabilitation needs	
4.	Continue to hold daily MDT's. Twice daily if required.	
5.	Act as single point of contact for families throughout admission.	
6.	Assist with discharge planning and onward care	
7.	Coordinate any repatriation of patients to other acute sites	
8.	Discontinue MDT's with agreement from therapy group once caseload within normal capacity.	

Appendix 2: Acute Mental Health Services Response

SCS Action Plan DRAFT 1.doc

Appendix 3: Adult Incident Flowchart



Flow chart A

Appendix 4: Paediatric Incident Flowchart

